

Lotline

A completed, documented
response to medical product recalls

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Draft: transfer to mandatory organizer template before submission

Can a clinic account for every potentially affected unit?

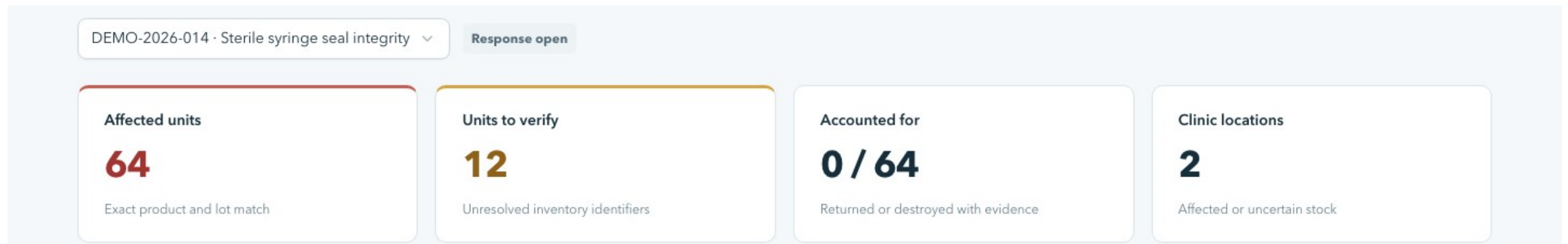
A notice names the product and lot.
The clinic must still find its stock
and complete the response.

- 01 Check each location
- 02 Investigate missing lots
- 03 Document every outcome

[FDA recall Z-2614-2026](#) required inventory checks, quarantine and replies even where no affected stock remained. Historical example.

One workspace for recall follow-through

Actual MVP with synthetic clinic stock



Unresolved inventory stays visible until an operator verifies the label.

Synthetic demonstration. No patient records. Operator-entered evidence does not independently certify physical stock.

Explicit rules preserve uncertainty

Control	Behavior
Product identity	One catalog + optional individual GTIN
Lot scope	Shared exact lot set, or explicit all-lots scope
Incomplete records	Keep missing identifiers unresolved
Quantity accounting	Record every affected unit using a permitted disposition

CSV and supported GS1 parsing. Ranges, wildcards and multi-product scopes require a different workflow.

Twelve unknown units keep the case open

Northstar stock	Units	Initial result
Riverside / SY2608-A	40	Matches reviewed scope
Hillview / SY2608-A	24	Matches reviewed scope
Hillview / missing lot	12	Needs investigation
Riverside / SY2608-B	30	Outside reviewed scope

76

affected units after
resolving the 12
unknown units to lot A

Completion requires
recorded
disposition for all 76 units.

Fictional product LL-SYR-10. Outside scope does not mean safe. Inventory figures illustrate the demo, not clinical outcomes.

A focused tool for small clinic groups

Who uses it

Operations leads who own the response across sites

Stockroom staff who identify units and record handling

What a pilot should measure

Time spent investigating a recall

Stock records fully accounted for

Completeness of the response record

First-market hypothesis: small US clinic groups handling device consumables. Benefits await field validation.

A subscription needs measurable value

Potential buyer

Clinic group or distributor sponsor

Pricing experiment

\$99 / group / month

3 sites included. \$19 per additional site.
Price and willingness to pay are unvalidated.

Sources: [ECRI](#) and [GHX](#) product descriptions. Proposed pricing only. No customers, revenue or partnerships claimed.

Existing products	Lotline hypothesis
ECRI: alerts and workflow	Simpler setup for small groups
GHX: inventory traceability	Work with basic stock exports
Recall feeds and watchlists	Charge for response accounting

A working MVP, with clear launch gates

Prototype workflow

Reviewed scope and stock import
Deterministic matching and resolution
Disposition accounting and receipt
Authenticated account workspaces
Persistent storage in D1

Before clinical deployment

Supervised operational validation
Shared organization roles and approvals
Authoritative notice ingestion
Retention and recovery controls
Security review and customer approval

48 deterministic core tests pass. Clinical validation, security certification and regulatory approval are not claimed.

A four-week pilot would test the business

3-5 sites

Historical or simulated notices
with an authorized operations lead

Success criteria to test

Zero false-clear outcomes in the
human-adjudicated evaluation set

At least 30% less median handling time

A buyer willing to continue at a paid price

Proposed targets only. No pilot or customer interviews completed. Measure onboarding effort and support cost alongside benefits.

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Project creator and product direction

A recall response should leave
no unknown unit hidden
behind a completed checkbox.

Developed with substantial AI assistance for research, software implementation, testing and submission drafting.